



Philippine Integrated Disease
Surveillance and Response

Case Investigation Form Acute Flaccid Paralysis



Name of DRU:				Type: ☐ RHU ☐ CHO ☐ Gov't Hospital ☐ Private Hospital ☐ Clinic				
Address:				☐ Gov't Laboratory ☐ Private Laboratory ☐ Airport/Seaport				
I. PATIENT INFORMATION:		Patient Number:	Patient's First Name		Middle Name	Last Name		
Sex: ☐ Male ☐ Female		Date of Birth: ____/____/____ MM DD YY		Age: _____		☐ Days ☐ Months ☐ Years		
Name of Parent/ Caregiver:				Complete Address:				
Contact No:								
Patient Admitted? ☐ Yes ☐ No				Date Admitted/ Seen/Consult		MM	DD	
Date of Report:		MM	DD	YY	Date of Investigation:		MM DD YY	
Name of Investigator:				Contact No:				
Is the case a member of an Indigenous Group? ☐ Y ☐ N If YES, Specify:								
II. CLINICAL DATA (Put a check [√] in the appropriate box)								
Previous Consultation	PRODROME	PARALYSIS	SITE OF FLACCID PARALYSIS	Sensory Status	Deep Tendon Reflexes	Motor Status		
Name and address of health facility 1: _____ Date of Visit: ____/____/____ Name and address of health facility 2: _____ Date of Visit: ____/____/____	Fever: ☐ Y ☐ N Cough: ☐ Y ☐ N Diarrhea/Vomiting: ☐ Y ☐ N Muscle pain: ☐ Y ☐ N Meningeal signs: ☐ Y ☐ N	Date onset: ____/____/____ Present at birth?: ☐ Y ☐ N Asymmetric?: ☐ Y ☐ N PROGRESSION Paralysis fully developed within 3 to 14 days from onset of illness? ☐ Y ☐ N Direction of paralysis: ☐ Ascending ☐ Descending	Right arm: ☐ Y ☐ N Left arm: ☐ Y ☐ N Right leg: ☐ Y ☐ N Left leg: ☐ Y ☐ N Breathing muscles: ☐ Y ☐ N Neck muscles: ☐ Y ☐ N Facial muscles: ☐ Y ☐ N Working Diagnosis:	_____ _____ _____ _____	_____ _____ _____ _____	_____ _____ _____ _____		
<i>NOTE: Instructions on the grading/scoring of the sensory status, deep tendon reflexes and motor status are presented at the back of this page.</i>								
III. EPIDEMIOLOGIC DATA								
History of neurologic disorder?: ☐ Y ☐ N If YES, specify disorder: _____								
Did the patient travel (>10 km from house) one month prior to illness? ☐ Y ☐ N								
If YES, specify place: _____ Date traveled: From ____/____/____ To ____/____/____								
Other AFP cases in patient's community within 60 days of patient's paralysis? ☐ Y ☐ N								
Does the patient had any history of injection, trauma and/ or animal bite ? ☐ Y ☐ N								
If YES, specify type _____								
Is there an Environmental Sample tested positive for WPV/ VDPV / Sabin-like 2 in the area? ☐ Y ☐ N IF YES, Specify the date: ____/____/____								
IV. IMMUNIZATION HISTORY								
Polio Vaccine given: ☐ Y ☐ N								
OPV Total Routine OPV doses received: ____ Date last OPV dose: ____/____/____ Total SIA OPV doses received: ____ Date last OPV dose: ____/____/____								
IPV Total IPV doses received: ____ Date last IPV dose: ____/____/____								
V. LABORATORY DATA								
Stool sample #	Collected?	If YES, date taken	Date sent to RITM	Date received RITM	Result		Amount of Stool (To be filled out by RITM)	Specimen Condition (To be filled out by RITM)
1	☐ Y ☐ N	____/____/____	____/____/____	____/____/____	☐ WPV ☐ Sabin-like ☐ VDPV ☐ NEG ☐ NPEV	If WPV, Sabin-like or VDPV: Specify: ☐ Type 1 ☐ Type 2 ☐ Type 3	_____ _____	No. of Ice packs: _____ Qty of Ice packs: ☐ Frozen ☐ Thawed but cold ☐ Warm Type of Container: _____
2	☐ Y ☐ N	____/____/____	____/____/____	____/____/____	☐ WPV ☐ Sabin-like ☐ VDPV ☐ NEG ☐ NPEV	If WPV, Sabin-like or VDPV: Specify: ☐ Type 1 ☐ Type 2 ☐ Type 3	_____ _____	Name of Courier: _____ RECEIVED BY: _____
VI. 60-DAY FOLLOW-UP								
Expected date of follow-up: ____/____/____ Follow-up done: ☐ Y ☐ N					If NO, reason for no P.E: _____			
If Yes, actual date of follow-up conducted: ____/____/____								
P.E. done? ☐ Y ☐ N					☐ Patient Died Date: ____/____/____			
Residual paralysis at 60 days?: ☐ Y ☐ N					☐ Lost to Follow-up			
If Yes, Specify: Flaccid/Floppy ☐ Spastic ☐					☐ OTHERS, Specify: _____			
Presence of Atrophy?: ☐ Y ☐ N					Note other observations: _____			
Site: RA: ☐ Y ☐ N LA: ☐ Y ☐ N								
RL: ☐ Y ☐ N LL: ☐ Y ☐ N								

Case Investigation Form

Acute Flaccid Paralysis

VII. CLASSIFICATION (TO BE FILLED UP BY THE EXPERT PANEL ONLY)		
FINAL CLASSIFICATION	CLASSIFICATION CRITERIA	FINAL DIAGNOSIS
☐ Confirmed wild polio ☐ Vaccine-derived paralytic polio (VDPV) ☐ Vaccine-associated paralytic polio (VAPP) ☐ Recipient VAPP ☐ Contact VAPP ☐ Polio compatible ☐ Discarded as Non-Polio Date classified: ____/____/____	☐ Laboratory ☐ Lost to follow-up ☐ Death ☐ With residual paralysis ☐ Without residual paralysis	Was this case considered as NOT AFP? ☐ Y ☐ N

AFP Case Definition:

- An AFP case is defined as a child less than 15 years of age presenting with recent or sudden onset of floppy paralysis or muscle weakness due to any cause, **OR**
- Any person of any age with paralytic illness if poliomyelitis is suspected by a clinician.

'Hot' or 'high risk' Case Description:

- A case that is considered highly suspected for being polio based on clinical data and with the following presenting characteristics:
 - Less than 5 years of age
 - Less than 3 OPV doses
 - Fever at onset of paralysis
 - Asymmetric paralysis
 - Rapid progression of paralysis (within 3 days)

And/or

- Has been in contact with or living in area with possible or recent Polio virus circulation.

Adequate Stool Definition:

- Two stool specimen (at least adult thumb size)
- Collected within 14 days from onset of paralysis
- With a collection interval of at least 24 hours

Grading/Scoring of Sensory Status, Deep Tendon Reflexes and Motor Status:

A. Sensory status is presented in percentage and categorized as follows:

- ≤25% = Absent
- ≥25% but <100% = Reduced
- 100% = Normal

B. Deep tendon reflexes are presented in (+) symbol and categorized as follows

- none or 0 = absent
- + = reduced
- ++ = normal
- +++ with/without clonus = increased or exaggerated

C. Motor Status is presented in fraction and categorized as follows:

- 0/5 = absent or no movement
- 1/5 to 3/5 = reduced movement (with movement but not against resistance or gravity)
- 4/5 to 5/5 = normal (movement with full resistance and against gravity)